

Droperidol

(Inapsine)

I. Actions/Effects:

- Powerful anti-psychotic (Dopamine receptor blocker)
- Anti-emetic
- Sedation

II. Indications:

- Nausea/Vomiting refractory to Ondansetron
- Migraines or Vertigo
- Agitated Schizophrenic or Psychotic episodes
- Chemical restraint
- Delirium Tremens/Alcohol Withdrawal (Only as required as an adjunct post Benzodiazepine administration)

III. Precautions:

- Obtain ECG as soon as possible.
- May be capable of potentiating CNS depressants such as anesthetics, opiates, other sedation medications, and alcohol.
- May cause dystonic reaction (Treat with Diphenhydramine per Behavioral Emergency COG)

IV. Contraindications:

- Parkinson's Disease
- Pregnancy
- Known EKG QTc over 500 ms

V. Administration:

- Agitation/Psychosis
 - 5 mg IV/IO
 - 10 mg IM
- Nausea/Vomiting
 - 0.625 mg IV/IO
 - 2.5 mg IM
 - May repeat dose x 1 in 30 min after confirming QTc < 450ms on EKG
- Pain Management for Headache
 - 1.25-2.5 mg IV/IO/IM
 - May repeat dose x 1 in 30 min after confirming QTc < 450ms on EKG
- **ELDERLY USE HALF DOSE**

VI. Onset:

- 3-10 minutes

VII. Duration:

- 2-4 hours

VIII. Side Effects / Special Notes:

- Monitor hemodynamic status and ECG closely (Watch for QT interval prolongation, Torsades de Pointes, or Ventricular arrhythmia)
- Epinephrine may paradoxically decrease the blood pressure in patients treated with droperidol due to the alpha-adrenergic blocking action of droperidol.